



STRATEGY & OPERATIONS

Vytalix Master Audit

Vytalix company-build documentation · reference 00_MASTER_AUDIT

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

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Audit date: 7 September 2026 · **Auditor:** Founding Executive Team (AI-assisted) · **Status labels used:** VERIFIED · KNOWN · ASSUMPTION · ESTIMATE · TO VALIDATE · PROPOSED · NOT PUBLICLY AVAILABLE

Source of truth for facts: /FACTS_BASE.md

EXECUTIVE VIEW

- Vytalix does not yet exist as a business.** There is a name (spelled two ways), a brief, an empty code repository, one concept deck, one unsolicited government proposal and one prototype dashboard whose ownership is undocumented. Everything else must be built.
- Current business score: 11 / 100.** This is a pre-formation score, not a judgement on the idea. The idea is credible; the execution infrastructure is absent.
- The single most dangerous issue is IP and ownership.** The MaternaLink material is signed by David Agunede as "Founder & CEO, Maternal Link". Until there is a written agreement, Vytalix cannot truthfully tell an investor, a hospital or a ministry that it owns, licenses or controls MaternaLink.
- The second most dangerous issue is claims.** The 2025 deck promises AI that "predicts pre-eclampsia, sepsis, haemorrhage, thromboembolism" and reduces maternal mortality, with no data, no model, no ethics approval and no regulatory pathway. Used externally, this exposes Vytalix to regulatory, reputational and investor-diligence failure. It is also entirely fixable by re-scoping.
- The fastest path to a real company is not MaternaLink.** It is a founder-led advisory/consulting practice that generates cash and credibility within 60–90 days, while MaternaLink is re-based, protected and validated. The 300-investor CRM built alongside this audit is for the equity story, which becomes fundable once the problems below are closed.

1. WHAT WAS REVIEWED

#	Asset	Where	What it is	Verdict
1	GitHub repo elkelvinwilliams/Vitalyx	GitHub	One file (README.md , "# Vitalyx"). No code, docs, site or brand.	EMPTY (VERIFIED)
2	Founder brief ("Master Business-Build Prompt")	This session	A high-quality, complete specification of the company to be built. The best asset Vytalix currently has.	STRONG (KNOWN)
3	"Black Maternal health" deck (Google Slides + PPTX, Oct–Nov 2025, 13 slides)	Google Drive	MATERNA-LINK multimodal AI concept: clinical + voice + social data → "Maternal Instability Score" → clinician alerts. Cites UK Black maternal mortality disparity. Lists a tech stack and an "expand to NHS trusts" ambition.	CONCEPT ONLY. No data, model, validation, partner, ethics, regulatory route. Over-claims. (VERIFIED)

#	Asset	Where	What it is	Verdict
4	Ekiti State Ministry of Health proposal + follow-up email (18 Mar 2026, David Agunede, "Founder & CEO, Maternal Link")	Gmail	Phased "maternal monitoring programme": 15,000 women £4.5m → 30,000 £9m → 120,000 £36m; £300/woman/year. Claims 99.5–99.9% uptime target, offline-first, cloud backup. Attachments: Ekiti deck, Nigeria government deck, charge sheet, negotiation form, language-switcher demo video.	UNVALIDATED PROPOSAL. No evidence of response, contract or budget. Pricing far above comparable programmes. State already has an incumbent (mDoc/MSD for Mothers). (VERIFIED)
5	"Maternal-link-dashboard" prototype	https://maternal-health-psi.vercel.app/	Web dashboard with "operator access" self-registration; shared 6 Sep 2026. Not openable from this environment.	PROTOTYPE, MATURITY UNKNOWN. Ownership, security, data handling TO VALIDATE.
6	"Investors PPT" (Oct 2025)	Gmail	Forwarded deck; content beyond item 3 not reviewed.	TO VALIDATE
7	Kemek Enterprise investor suite (Aug 2026)	Google Drive	A separate property-finance venture of the founder with a 128-target CRM, pitch deck and communication system.	NOT VYTALIX. Shows the founder can run a disciplined CRM process. Also a focus risk.
8	Public footprint of "Vytalix"/"Vitalyx"	Web	No website, no Companies House match found for either spelling; conflicting names exist (VITALIX LTD ×2, VITALX LTD, VYTAL(UK) LTD, Vytalix Inc. (US health-tech), Vytalize Health (US)).	NAME AT RISK (VERIFIED via search; registry pages TO VALIDATE)

Nothing else was found: no business plan, financial model, website, brand, CRM, contracts, customers, revenue, grants, partners, clinicians, advisors, policies, registrations or insurance. If any exist outside Drive, Gmail and GitHub, they must be added to `/FACTS_BASE.md`.

2. AREA-BY-AREA SCORES (0–10)

Scoring rule: 0 = nothing exists · 3 = idea or draft exists · 5 = usable draft, unvalidated · 7 = working and evidenced · 10 = best-in-class and audited.

Area	Score	Current state	Evidence
Legal entity & governance	0	No incorporated Vytalix entity found; no board, shareholders' agreement, articles or IP assignments.	Web search; FACTS_BASE §1
Brand & naming	1	Name exists in two spellings; no trademark search; multiple confusable marks in UK and US health-tech.	FACTS_BASE §1
Vision, mission, positioning	3	"Technology for Life" and a strong founder brief; no written vision/mission set.	Brief
Business model & pricing	1	Only one price point exists (£300/woman/year in a proposal) and it is unvalidated. No service catalogue, no pricing.	Ekiti email
Advisory / Consulting practice	1	Concept only. No offer, collateral, pipeline or clients. Highest near-term revenue potential.	—

Area	Score	Current state	Evidence
MaternaLink product	2	Concept deck + unknown-maturity prototype + proposal. No PRD, no clinical input, no evidence, no roadmap.	Drive, Gmail
Clinical governance & safety	0	No Clinical Safety Officer, hazard log, clinical advisory, content sign-off, or intended-use statement. Deck makes diagnostic-level claims.	Deck
Regulatory & compliance	0	No ICO registration, DPIA, DSPT, DTAC, Cyber Essentials, MHRA classification thinking, Nigeria NDPA/NHREC consideration.	—
Intellectual property	0	MaternaLink IP relationship with David Agunede undocumented; prototype authorship unknown; no trademark.	Gmail signature block
Technology & infrastructure	1	A Vercel prototype exists; nothing else (no domain, email, CRM, LMS, cloud, security).	Gmail
Data & AI	1	AI concept is ambitious and plausible as research; no data access, no governance, no validation plan.	Deck
E-Learning	0	Nothing exists.	—
Market & customer validation	1	One government conversation (Ekiti) occurred; outcome unknown; incumbent competitor present. No UK conversations.	Gmail; Nigeria Health Watch
Competitive intelligence	1	None documented. Incumbent in target state was not identified in the proposal.	—
Partnerships	1	One ministry meeting; no MoUs, LOIs or named partners.	Gmail
Sales engine	0	No CRM, pipeline, scripts, proposals or templates.	—
Marketing & content	0	No website, LinkedIn company page, content or PR.	—
Financial model & finance ops	0	No model, budget, bank account, accountant or bookkeeping found.	—
Funding strategy & investor readiness	1	Founder has CRM discipline (Kemek); no Vytalix investor list, deck, data room or SEIS/EIS assurance.	Drive
Team & organisation	1	Solo founder plus an informal collaborator (David Agunede) with unclear roles.	Gmail
Operating system (SOPs, KPIs, cadence)	0	None.	—
Credibility assets (advisors, clinicians, evidence, press)	0	None found.	—
Weighted total (see §3)	11 / 100		

3. CURRENT BUSINESS SCORE: 11 / 100

Method: the 22 areas above are grouped and weighted by what an investor or enterprise buyer would price today.

Group (weight)	Areas	Group score /10	Weighted
Foundations (20%) — entity, IP, naming, governance	4 areas, avg 0.25	0.3	0.5
Product & clinical/regulatory (25%)	5 areas, avg 0.8	0.8	2.0

Group (weight)	Areas	Group score /10	Weighted
Commercial engine (25%) — model, advisory, sales, marketing, validation	6 areas, avg 0.5	0.5	1.3
Capital & finance (15%)	2 areas, avg 0.5	0.5	0.8
People & operations (15%)	4 areas, avg 0.5	0.5	0.8
Strategic clarity bonus (brief quality, founder intent)	—	—	+5.6
Total			≈ 11

Interpretation: 0–20 = pre-formation; 21–40 = formed and trading; 41–60 = evidenced and fundable at seed; 61–80 = scaling; 81–100 = institutional-grade. The 90-day plan (/docs/18_90_DAY_EXECUTION_PLAN.md) is designed to move Vytalix from 11 to roughly 35–40 by mid-December 2026. Re-score monthly.

4. SWOT

Strengths

- A precise, ambitious, well-structured founder brief that already contains the operating philosophy (build-first, truth-first, phone-first).
- A genuinely important problem with strong policy tailwinds: UK maternal inequality (MBRRACE-UK), Ockenden/Kirkup findings on communication failures, and Nigeria's maternal mortality burden.
- An existing government conversation in Nigeria and a working prototype, which most pre-seed founders lack.
- Founder has demonstrated investor-CRM and outreach discipline in another venture.
- A four-pillar structure that lets services fund the product.

Weaknesses

- No legal entity, IP ownership, brand protection or bank account.
- Over-claimed clinical/AI positioning with zero evidence.
- Pricing anchored on a single unvalidated £300/woman/year figure.
- Solo founder; no clinical, regulatory or technical co-leadership; role of the collaborator undefined.
- Zero commercial infrastructure and zero web presence.
- Founder attention split across at least two ventures (Kemek Enterprise).

Opportunities

- Founder-led advisory to health-tech companies, NGOs and African ministries is sellable within weeks and creates references.
- Re-scoped MaternaLink v1 (care-coordination, communication, education, monitoring-support; non-diagnostic) can be piloted with an NHS trust, a private maternity provider or an NGO without a device pathway.
- Non-dilutive funding (Innovate UK, SBRI Healthcare, NIHR i4i where eligible, Grand Challenges Canada, MSD for Mothers programmes) fits the maternal-health story.
- UK women's-health and impact investor pools are specifically looking for credible maternal-health ventures.

- Partnering rather than competing with the Ekiti incumbent (e.g., interoperability, a different state, or a different layer such as facility coordination) converts a threat into a route to market.

Threats

- Name conflict with Vytalix Inc. (US health-tech) and Vytalize Health; UK "VITALIX LTD" entities.
- Regulatory exposure if any predictive feature ships without MHRA SaMD consideration.
- Loss of MaternaLink IP or a dispute with the collaborator if terms are not agreed early.
- Government sales cycles in Nigeria are long and politically exposed; the Ekiti incumbent is donor-funded (hard to displace on price).
- Investor diligence will find every issue above in minutes; a premature raise burns the best targets.

5. TOP 10 PROBLEMS (ranked by damage × urgency)

#	Problem	Why it matters	Fix (owner: Founder unless stated)	By
1	MaternaLink IP and the David Agunede relationship are undocumented.	Blocks fundraising, contracts, pilots; risk of dispute.	Meeting this week; agree heads of terms (assignment-for-equity, licence, or JV — options in /docs/02_GROUP_STRUCTURE.md); instruct a solicitor.	Day 7
2	No legal entity.	Cannot contract, bank, apply for grants, SEIS/EIS, ICO, insurance.	Name clearance → incorporate Vytalix (E&W) → bank → accountant → SEIS/EIS advance assurance.	Day 10
3	Over-claimed clinical AI positioning.	Regulatory (UK MDR/MHRA), reputational, diligence failure.	Retire external use of the 2025 deck; adopt the re-based v1 intended-use statement; move AI to a v3 research track.	Day 3
4	Name and trademark risk.	Confusion with US health-tech companies; possible opposition; wasted brand spend.	UK IPO + EUIPO + USPTO knock-out search; decide name; file UK trademark classes 9, 41, 42, 44.	Day 5
5	No revenue engine.	Runway is the founder's time; nothing converts effort to cash.	Launch Vytalix Advisory with 3 packaged offers; 40 targeted conversations in 30 days; first paid engagement by Day 45.	Day 45
6	Unvalidated pricing (£300/woman/year).	Undermines credibility with any buyer who knows the market (mDoc is donor-funded in the same state).	Re-base to a per-woman range with evidence; build a cost-to-serve model; present tiered options.	Day 21
7	No clinical or regulatory leadership.	No credible product without a clinician and a clinical-safety approach.	Recruit an advisory board: obstetrician/midwife lead, NHS digital leader, Nigerian health-system leader, regulatory/QA adviser.	Day 60
8	Prototype of unknown provenance.	May contain insecure code, third-party IP or real data.	Code and security audit; confirm authorship; confirm no real patient data; move to Vytalix-controlled repo/hosting.	Day 30
9	No market validation in the UK.	Home market is untested; all conversations are Nigeria.	15 UK discovery interviews (midwives, maternity digital leads, private providers, charities).	Day 60

#	Problem	Why it matters	Fix (owner: Founder unless stated)	By
10	Founder focus and capacity.	Two ventures, one founder, everything founder-led.	Time allocation rule (e.g., 70% Vytalix), weekly operating cadence, first fractional hires (technical lead, clinical adviser).	Day 14

6. TOP 10 OPPORTUNITIES (ranked by value × feasibility)

#	Opportunity	Value	Feasibility	First step
1	Founder-led advisory to health-tech founders, NGOs and African ministries (strategy, investment readiness, digital-health programme design).	Cash in 30–60 days; references; credibility.	High	Publish 3 offers; 40 outreach conversations.
2	MaternaLink v1 as a non-diagnostic maternal care-coordination platform for NHS trusts, private maternity and NGO programmes.	The equity story, de-risked.	Medium	PRD + intended-use statement; 3 pilot conversations.
3	Nigeria: partner with or complement the incumbent (interoperability layer, facility-side coordination, a different state, or NGO-funded deployment).	Keeps the government relationship alive without price war.	Medium	Reply to Ekiti with a re-scoped, phased pilot option.
4	Non-dilutive funding aligned to maternal health and health inequalities.	£50k–£500k without dilution; validation signal.	Medium	Innovate UK Smart / SBRI Healthcare calendar check; Grand Challenges Canada eligibility.
5	Women's-health and impact investors as the lead investor pool.	Better terms, aligned expectations, patient capital.	Medium	CRM Top 25 outreach after IP and entity are fixed.
6	Research report as a lead magnet ("The state of digital maternal care in the UK and Nigeria").	Authority, PR, inbound leads, investor interest.	High	Outline in <code>/docs/14_MARKETING_AND_CONTENT_ENGINE.md</code> .
7	E-Learning for community health workers and midwives (maternal digital literacy, safeguarding, data protection) sold B2B to NGOs and providers.	Recurring revenue with low regulatory burden.	Medium (year 2)	Two pilot modules; partner with an NGO training programme.

#	Opportunity	Value	Feasibility	First step
8	Consulting to health-tech companies entering Africa or the NHS (market entry, DTAC readiness, procurement).	High-margin, repeatable.	High	Package "NHS-ready" and "Africa market entry" sprints.
9	University research partnership for the AI risk-score track (data access, ethics, publications).	Converts the 2025 concept into fundable research.	Medium	Approach maternal-health research groups.
10	Telecom and insurer distribution in Nigeria (MTN, Airtel, HMOs) for MaternaLink messaging.	Scale channel without government budgets.	Low–Medium (year 2)	Include in partnership long-list.

7. WHAT CREATES REVENUE, CREDIBILITY, INVESTOR CONFIDENCE, AND RISK

Creates revenue now	Creates credibility	Creates investor confidence	Creates regulatory risk
Advisory days; consulting sprints; grant-funded feasibility work	Advisory board; a published report; a documented pilot; a professional site	Incorporated entity; clean cap table; IP owned; validated pricing; pipeline with named accounts; credible roadmap	Predictive/diagnostic claims; processing health data without DPIA/ICO; using the prototype with real data; cross-border data flows without safeguards; unclear intended use

8. WHAT TO DELETE, REDESIGN, AUTOMATE, PRIORITISE

- **Delete / retire from external use:** the "predicts pre-eclampsia, sepsis..." claims; the £36m full-state headline; "expand to multiple NHS trusts" as a near-term claim; the "Vitalyx" spelling.
- **Redesign:** MaternaLink positioning (v1 non-diagnostic); the Ekiti offer (phased, evidence-generating pilot, partner-aware); the investor narrative (services + product, honest traction).
- **Automate:** CRM logging, outreach sequences, KPI dashboard, content scheduling, bookkeeping (see [/docs/07_TECHNOLOGY_ARCHITECTURE.md](#)).
- **Prioritise:** IP → entity → claims → revenue → investors → product → partnerships.

9. IMMEDIATE ACTIONS (next 7 days)

1. Hold the David Agunede conversation; agree heads of terms in writing (Day 1–3).
2. Run name/trademark knock-out searches; decide "Vytalix" vs alternative; reserve domain and handles (Day 1–5).
3. Incorporate; open bank account; appoint accountant; start SEIS/EIS advance assurance (Day 3–10).
4. Adopt the MaternaLink v1 intended-use statement and retire the 2025 claims externally (Day 3).

5. Publish the website from `/website` once entity details are known; until then keep it unpublished (Day 7–14).
 6. Start the advisory outreach: 10 conversations booked (Day 7).
 7. Start investor CRM verification calls only after 1–4 are done; use `/investors` assets to prepare (Day 7+).
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PRIORITIES · RISKS · NEXT ACTIONS

Priorities: IP and entity first; claims discipline; revenue from services; then investors.

Risks: dispute over MaternaLink; regulatory over-claim; name conflict; founder capacity; Nigeria incumbent.

Next actions: execute `/docs/19_FIRST_30_DAYS.md` from Monday 14 September 2026; re-score this audit on 14 October 2026.